**Treatment scope · NCCP 00259** Lung monotherapy context only. The breast guide and its evidence remain separate. Discuss the exact disease setting and alternatives with the treating team.

NCCP 00259 · VERSION 6

# Vinorelbine capsules · non-small-cell lung cancer

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Discussion aid: review with the patient and complete the approved local consent process.

## Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

## Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

## Selected material risks

☐ Low blood counts, infection and bleeding

☐ Constipation or bowel obstruction

☐ Incorrect weekly dosing or damaged capsules

☐ Nausea, vomiting and fatigue

☐ Nerve effects and uncommon lung/heart toxicity

Discuss the disease-control aim, likely benefit and uncertainty, other lung-cancer treatments or symptom-focused care, and the burden of visits and tests. Review low blood counts and infection, nausea/vomiting, fatigue, constipation/ileus, nerve symptoms, hair changes and uncommon serious lung/heart/allergic effects. Discuss once-weekly capsule safety, food, anti-sickness treatment, handling, dose escalation and no repeat dose after vomiting. Discuss fertility preservation and pregnancy planning. Do not breastfeed. Confirm the exact product contraception advice: HPRA recommends contraception during treatment and for 7 months afterwards for women, and 4 months for men.

## Individual benefit, alternatives and concerns

## Agreed checks and next review

Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.